

Staffordshire Integrated Diabetic Foot Pathway



Midlands Partnership
NHS Foundation Trust
A Keele University Teaching Trust

Integrated pathway designed to deliver multi-professional care with step up/step down/shared care across organisations

EMERGENCY ACTION REQUIRED

SEVERE INFECTION # (See table below)

ABSCCESS/WET GANGRENE/LIMB THREATENING INFECTION
ACUTE CRITICAL LIMB ISCHAEMIA (SUDDEN ONSET COLD, WHITE, PAINFUL FOOT/LEG)

Sepsis OR acute critical ischaemia call 999

Limb threatening infection/abscess/wet gangrene

Contact Vascular on Call via switchboard at Vascular Hub Hospital to discuss admission & inform primary Care. Royal Stoke 01782 715444. Russells Hall 01384 456111. Heartlands 0121 424 2000 . Royal Derby 01332 340131

Hospital discharge to hospital Diabetic Foot Multidisciplinary Team who will co-ordinate follow up and agree care plan with MDT, primary care and community services

URGENT ACTION REQUIRED

NEW DIABETIC FOOT ULCER
NON-EMERGENCY FOOT ULCER COMPLICATIONS
INCLUDING MILD & MODERATE INFECTION#
(See table below)

Urgent referral to Diabetic Foot Multidisciplinary Team member within 1 working day for specialist assessment & care planning Complete referral form and email to

- PodiatryReferralsNorth@mpft.nhs.uk (North Staffs/Stoke on Trent), Newcastle, Leek & Moorlands, The Haywoods Hospital)
- apptbookingcentre@mpft.nhs.uk (Cannock/Stafford/Seisdon/South East Staffs)
- Podiatrystgeorges@mpft.nhs.uk for St Georges Hospital
- bhft.diabetescentre@nhs.net (East Staffs)

Remember first line management of diabetic foot infection

INFECTION

Immediately swab ulcer and prescribe empiric antibiotics. Refer to Staffordshire Antimicrobial Prescribing Guidance 2019 and **infection severity grading # (see table right)**

SEVERITY GRADING OF DIABETIC FOOT INFECTION#

Clinical Criteria	Severity
No signs of infection	Uninfected
Superficial wound with one or more signs & symptoms of inflammation listed below Redness, pain, heat, swelling PLUS cellulitis extending >0.5- 2cm around the wound	Mild
As for mild criteria, PLUS one or more of the following; Cellulitis extending >2cm around the wound, lymphangitic streaking, localised dry gangrene, deep tissue involvement. Patient is systemically well and metabolically stable	Moderate
As for moderate criteria PLUS systemic features of infection/sepsis such as raised/low temperature, low BP, heart rate >90bpm, malaise	Severe

Stabilised with care plan shared care with community and primary care partners

Diabetic Foot Multidisciplinary Team will include
Diabetes Team/Dietician/Orthotist/
Vascular/Orthopaedic/Microbiology / Radiology/
Advanced Podiatrist/Amputee Rehab

Diabetic Foot MDT members may be co-located or work collaboratively across organisations

URGENT ACTION REQUIRED

SUSPECTED ACUTE CHARCOT (UNEXPLAINED HOT SWOLLEN DIABETIC FOOT WITH OR WITHOUT FOOT PAIN)

Urgent referral for orthopaedic surgeon opinion **PLUS** Diabetic Foot Multidisciplinary Team referral (as per 'new diabetic foot ulcer') within 24 hours

Advise to maximise rest, elevate the affected leg and provide Charcot patient information